

HOSOLVON TABLET 8 mg (GREEN)

VIHOSXX-M3

DESCRIPTION

Round, green uncoated tablet, shallow convex faces, "HD" embossed and scored on the same face.

COMPOSITION

Bromhexine hydrochloride 8 mg/tablet.

PHARMACODYNAMICS

Bromhexine is an expectorant/mucolytic agent. The drug is a benzylamine derivative (2-amino-3, 5-dibromo-N-cyclohexyl N-methylbenzylamine hydrochloride) and also a derivative of valicic acid and adhatodic acid, alkaloids obtained from the plant *Adhatoda vasica*.

Following oral administration, bromhexine has increased sputum volume and reduced the viscosity of bronchial secretions in chronic bronchitis patients. The drug has been reported to induce hydrolytic depolymerization of mucoprotein fibers and stimulate activity of the ciliated epithelium. An increase in lysosomal activity facilitated by bromhexine has been postulated. Improvements in pulmonary function in bronchitis patients appear secondary to easier expectoration.

An effect of bromhexine on increasing sputum concentrations of various antibiotics (eg, oxytetracycline, erythromycin, ampicillin, amoxicillin) has also been reported. However, some of these effects (exocrine stimulation, increased sputum concentrations) have not been confirmed in some studies.

It has been suggested that a metabolite of bromhexine, ambroxol, may contribute to enhanced secretion from exocrine glands during bromhexine administration.

PHARMACOKINETICS

Absorption

Oral, well absorbed.

Rapidly absorbed from the gastrointestinal tract.

Peak plasma concentrations occur after about 1 hour following oral administration.

Metabolism

Bromhexine undergoes extensive first-pass metabolism in the liver. Ambroxol is a metabolite of bromhexine.

Distribution

It is widely distributed to body tissues.

Bromhexine is highly bound to plasma proteins.

Bromhexine crosses the blood-brain barrier and small amounts cross the placenta.

Excretion

Bromhexine is excreted primarily in the urine as metabolites. Only small amounts appear as unchanged drug.

About 85 to 90% of a dose is excreted in the urine mainly as metabolites.

Approximately 70% of an oral dose of bromhexine has been recovered in the urine within 24 hours.

Other excretion: faeces, 4%

Elimination Half-life

It has a terminal elimination half-life of 13 to 40 hours.

INDICATION

Secretolytic therapy in acute and chronic bronchopulmonary diseases associated with abnormal mucus secretion and impaired mucus transport.

DOSAGE AND ADMINISTRATION

Oral.

Adults : Orally, 1 to 2 tablets 3 times daily.
Children 6 - 12 years : Orally, 1/2 tablet 3 times daily.

CONTRAINDICATIONS

Hypersensitivity to bromhexine or other components of the formulation.

WARNINGS AND PRECAUTIONS

- Patients being treated with bromhexine should be notified of an expected increase in the flow of secretions.
- The use of Bromhexine without medical supervision is not intended in patients who suffer from such condition chronically.
- Medical advice should be sought if the symptoms last longer than 14 days and/or if the symptoms increase in spite of treatment with bromhexine.
- Since mucolytics may disrupt the gastric mucosal barrier bromhexine should be used with care in patients with a history of peptic ulcer disease (gastric ulcer).
- Care is also advisable in asthmatic patients.
- Clearance of bromhexine or its metabolites may be reduced in patients with severe hepatic or renal impairment.
- Bromhexine may increase the amount of antibiotic penetration. Antibiotics are medicines used to treat infections.

INTERACTIONS WITH OTHER MEDICAMENTS

Bromhexine may increase the concentration of concurrently administered antibiotics in bronchial secretions. No clinically relevant interactions with other medications have been reported.

PREGNANCY AND LACTATION

Pregnancy

There is no data on the use of bromhexine in pregnant women. The effects, if any, on the developing fetus are unknown. Until more information is available, bromhexine should only be used during pregnancy if the maternal condition justifies the potential risk to the fetus.

Lactation

Available evidence and/or expert consensus is inconclusive or is inadequate for determining infant risk when used during breastfeeding. Weigh the potential benefits of drug treatment against potential risks before prescribing this drug during breastfeeding.

ADVERSE EFFECTS

Dermatologic Effects: Skin rash, urticaria.

Gastrointestinal Effects: Nausea, epigastric pain, vomiting, and diarrhoea.

Hepatic Effects: Transient elevations in serum aminotransferase levels.

Neurologic Effects (Central nervous system): Dizziness and headache

Renal Effects: Nocturnal enuresis

Others:

- anaphylaxis.
- bronchospasm; difficulty in breathing.
- angioedema; swelling of the face, lips, mouth, tongue or throat which may cause difficulty swallowing or breathing.

OVERDOSE AND TREATMENT

No symptoms of overdosage have been reported in man to date. If they occur, supportive and symptomatic treatment should be provided.

Note: The information given here is limited. For further information consult your doctor or pharmacist.

Storage:

Store below 30°C. Protect from light and moisture.

Presentation/Packing:

Tablet 8 mg x 100's, 1000's, blister of 3 x 10's, 10 x 10's.

Product registration holder: HOVID Bhd.

121, Jalan Tunku Abdul Rahman,
30010 Ipoh, Malaysia.

Manufactured by: HOVID Bhd.

Lot 56442, 7 1/2 Miles, Jalan Ipoh/Chemor,
31200 Chemor, Malaysia.

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